Cemiplimab for advanced cutaneous squamous cell cancer

Your diagnosis and treatment setting NCCP 00812b: metastatic or locally advanced cutaneous squamous cell carcinoma when curative surgery or curative radiotherapy is not appropriate. This guide covers skin squamous cell carcinoma (SCC), not the separate cervical-cancer indication.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cemiplimab treatment

Active medicines: Cemiplimab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Routine antiemetic prophylaxis or premedication is not usually required by the source. Provide an immune-toxicity contact/alert plan; steroid or hormone treatment is prescribed only for the individual indication.
- Use effective contraception during cemiplimab treatment and for at least four months after the last dose. Discuss fertility before treatment, and follow the medicine-specific breastfeeding restrictions. Report a possible pregnancy promptly.
- Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____